

j) **Route of Administration**

Method of administration:

Decitabine is administered by intravenous infusion. A central venous catheter is not required.

k) **Contraindications:**

Hypersensitivity to Decitabine or to any of the excipients.
Lactation.

l) **Warnings & Precautions:**

Myelosuppression:

Myelosuppression and complications of myelosuppression, including infections and bleeding that occur in patients with AML may be exacerbated with Decitabine treatment. Therefore patients are at increased risk for severe infections (due to any pathogen such as bacterial, fungal and viral), with potentially fatal outcome. Patients should be monitored for signs and symptoms of infection and treated promptly.

Respiratory, thoracic and mediastinal disorders:

Cases of interstitial lung disease (ILD) (including pulmonary infiltrates, organising pneumonia and pulmonary fibrosis) without signs of infectious aetiology have been reported in patients receiving Decitabine. Careful assessment of patients with an acute onset or unexplained worsening of pulmonary symptoms should be performed to exclude ILD. If ILD is confirmed, appropriate treatment should be initiated.

Hepatic impairment:

Use in patients with hepatic impairment has not been established. Caution should be exercised in the administration of Decitabine to patients with hepatic impairment and in patients who develop signs or symptoms of hepatic impairment. Liver function tests should be performed prior to initiation of therapy and prior to each treatment cycle, and as clinically indicated

Renal impairment:

Caution should be exercised in the administration of Decitabine to patients with severe renal impairment (Creatinine Clearance [CrCl] < 30 ml/min).

Cardiac disease:

Patients with a history of severe congestive heart failure or clinically unstable cardiac disease were excluded from clinical studies and therefore, the safety and efficacy of Decitabine in these patients has not been established. Cases of cardiomyopathy with cardiac decompensation, in some cases reversible after treatment discontinuation, dose reduction or corrective treatment, have been reported in the postmarketing setting. Patients, especially those with cardiac disease history, should be monitored for signs and symptoms of heart failure.

Differentiation syndrome:

Cases of differentiation syndrome (also known as retinoic acid syndrome) have been reported in patients receiving Decitabine. Differentiation syndrome may be fatal (see section o). Treatment with high-dose IV corticosteroids and haemodynamic monitoring should be considered at first onset of symptoms or signs suggestive of differentiation syndrome. Temporary discontinuation of Decitabine should be considered until resolution of symptoms and if resumed, caution is advised.

Excipients:

This medicine contains 0.5 mmol potassium per vial. After reconstitution and dilution of the solution for intravenous infusion, this medicine contains less than 1 mmol (39 mg) of potassium per dose, i.e. essentially 'potassium-free'.

This medicine contains 0.29 mmol (6.67 mg) sodium per vial. After reconstitution and dilution of the solution for intravenous infusion, this medicine contains between 13.8 mg-138 mg (0.6-6 mmol) sodium per dose (depending on the infusion fluid for dilution), equivalent to 0.7-7% of the WHO recommended maximum daily intake of 2 g sodium for an adult.

m) **Interactions with Other Medicaments**

No formal clinical drug interaction studies with Decitabine have been conducted. There is the potential for a drug-drug interaction with other agents which are also activated by sequential phosphorylation (via intracellular phosphokinase activities) and/or metabolized by enzymes implicated in the inactivation of Decitabine (e.g., cytidine deaminase). Therefore, caution should be exercised if these drugs are combined with Decitabine.

Impact of co-administered medicinal products on Decitabine:

Cytochrome (CYP) 450-mediated metabolic interactions are not anticipated as Decitabine metabolism is not mediated by this system but by oxidative deamination.

Impact of Decitabine co-administered medicinal products:

Given its low in vitro plasma protein binding (< 1%), Decitabine is unlikely to displace co-administered medicinal products from their plasma protein binding. Decitabine has been shown to be a weak inhibitor of P-gp mediated transport in vitro and is therefore also not expected to affect P-gp mediated transport of co-administered medicinal products.

n) **Pregnancy and Lactation**

Contraception in males and females

Women of childbearing potential must use effective contraceptive measures and avoid becoming pregnant while being treated with Decitabine. The time period following treatment with Decitabine where it is safe to become pregnant is unknown. Men should use effective contraceptive measures and be advised to not father a child while receiving Decitabine, and for 3 months following completion of treatment.

The use of Decitabine with hormonal contraceptives has not been studied.

Pregnancy:

There are no adequate data on the use of Decitabine in pregnant women. Studies have shown that Decitabine is teratogenic in rats and mice. The potential risk for humans is unknown. Based on results from animal studies and its mechanism of action, Decitabine should not be used during pregnancy and in women of childbearing potential not using effective contraception. If Decitabine is used during pregnancy, or if a patient becomes pregnant while receiving this medicinal product, the patient should be apprised of the potential hazard to the foetus.

Lactation:

It is not known whether Decitabine or its metabolites are excreted in breast milk. Decitabine is contraindicated during lactation; therefore if treatment with Decitabine is required, breast-feeding must be discontinued.

Fertility:

In non-clinical animal studies, Decitabine alters male fertility and is mutagenic. Because of the possibility of infertility as a consequence of Decitabine therapy, men should seek advice on conservation of sperm and female patients of childbearing potential should seek consultation regarding oocyte cryopreservation prior to initiation of treatment with Decitabine.

o) **Side effects**

Haematologic adverse drug reactions

The most commonly reported haematologic adverse drug reactions associated with Decitabine treatment included febrile neutropaenia, thrombocytopenia, neutropaenia, anaemia and leukopaenia.

Serious bleeding-related adverse drug reactions, some of which lead to a fatal outcome, such as central nervous system (CNS) haemorrhage and gastrointestinal (GI) haemorrhage, in the context of severe thrombocytopenia, were reported in patients receiving Decitabine.

Haematological adverse drug reactions should be managed by routine monitoring of complete blood counts and early administration of supportive treatments as required. Supportive treatments include, administration of prophylactic antibiotics and/or growth factor support (e.g., G-CSF) for neutropaenia and transfusions for anaemia or thrombocytopenia according to institutional guidelines. For situations where Decitabine administration should be delayed.

Infections and infestations adverse drug reactions

Serious infection-related adverse drug reactions, with potentially fatal outcome, such as septic shock, sepsis, pneumonia, and other infections (viral, bacterial and fungal) were reported in patients receiving Decitabine.

Gastrointestinal disorders

Occurrences of enterocolitis, including neutropaenic colitis, caecitis have been reported during treatment with Decitabine. Enterocolitis may lead to septic complications and may be associated with fatal outcome.

Respiratory, thoracic and mediastinal disorders

Cases of interstitial lung disease (including pulmonary infiltrates, organising pneumonia and pulmonary fibrosis) without signs of infectious aetiology have been reported in patients receiving Decitabine.

Differentiation syndrome

Cases of differentiation syndrome (also known as retinoic acid syndrome) have been reported in patients receiving Decitabine. Differentiation syndrome may be fatal and symptoms and clinical findings include respiratory distress, pulmonary infiltrates, fever, rash, pulmonary oedema, peripheral oedema, rapid weight gain, pleural effusions, pericardial effusions, hypotension and renal dysfunction. Differentiation syndrome may occur with or without concomitant leucocytosis. Capillary leak syndrome and coagulopathy can also occur.

Neoplasms benign, malignant and unspecified (including cysts and polyps).

Frequency 'very rare': Differentiation syndrome.

Pediatric population

The safety assessment in pediatric patients is based on the limited safety data to evaluate pharmacokinetics, safety and efficacy of Decitabine in pediatric patients (aged 1 to 14 years) with relapsed or refractory AML. No new safety signal was observed in this pediatric study.

p) **Symptoms and Treatment of Over dose**

There is no direct experience of human overdose and no specific antidote. However, early clinical study data in published literature at doses greater than 20 times higher than the current therapeutic dose, reported increased myelosuppression including prolonged neutropaenia and thrombocytopenia. Toxicity is likely to manifest as exacerbations of adverse drug reactions, primarily myelosuppression. Treatment for overdose should be supportive.

q) **Effects on Ability to Drive and Use Machine**

Decitabine may have moderate influence on the ability to drive and use machines. Patients should be advised that they may experience undesirable effects such as anemia during treatment. Therefore, caution should be recommended when driving a car or operating machines.

r) **Preclinical Safety Data**

Not Applicable.

s) **Instruction for use**

Instructions for Intravenous Administration

This medicinal product is for single use only. Skin contact with the solution should be avoided and protective gloves must be worn. Standard procedures for dealing with anticancer agents should be adopted. The powder should be aseptically reconstituted with 10 mL of Sterile Water for Injection. Upon reconstitution, each mL contains approximately 5.0 mg of Decitabine at pH 6.7 to 7.3. Immediately after reconstitution, the solution should be further diluted with 0.9% Sodium Chloride Injection or 5% Dextrose Injection to a final drug concentration of 0.15 to 1.0 mg/mL. Unless used within 15 minutes of reconstitution, the diluted solution must be prepared using cold (2°C to 8°C) infusion fluids and stored at 2°C to 8°C for up to a maximum of 4 hours until administration. Any unused product or waste material should be disposed of in accordance with local requirements.

t) **Storage conditions**

Store below 30°C.

After reconstitution: Unless used within 15 minutes of reconstitution, the diluted solution must be prepared using cold (2°C to 8°C) infusion fluids and stored at 2°C to 8°C for up to a maximum of 4 hours until administration.

u) **Shelf life**

36 months

After reconstitution:

Unless used within 15 minutes of reconstitution, the diluted solution must be prepared using cold (2°C to 8°C) infusion fluids and stored at 2°C to 8°C for up to a maximum of 4 hours until administration.

v) **Therapeutic Code**

L01BC08

w) **How Supplied**

DECISPAL-P 50

Decitabine powder for concentrate for solution for Infusion, 50 mg/vial

Single use vial, individually packed in a carton.

x) **Name and address of manufacturer/ product registration holder**

MANUFACTURED BY:

 **SP Accure Labs Pvt. Ltd.**

Plot No.12, Biotech Park, Phase 2, Lalgadi Malakpet,
Shameerpet(M), Medchal Dist., Telangana, 500 101, India.

Product registration holder

Healol Pharmaceuticals Sdn.Bhd.
74-3, Jalan Wangsa Delima 6,
KLSC Wangsa Maju,
53300 Kuala Lumpur, Malaysia

y) **Date of revision of PI**

29.04.2022